

ENGLISH FOR WORK / SEPTEMBER 2026

Home Care & Caregiving English



Conversation lab

8 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Home care aides, personal care assistants, and caregivers.

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

- 01 Sorting out a duplicated request
- 02 A preference that changes mid-visit
- 03 Correcting a report before it spreads
- 04 Helpful without stepping outside the assignment
- 05 A disagreement about the meal record
- 06 An authorized contact is not an unlimited audience
- 07 A handoff after a last-minute change
- 08 Explaining a service problem without blaming the client

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

FULL DIALOGUE 01 / 12 SPEAKING TURNS

Sorting out a duplicated request

A caregiver and coordinator review a client's concern that laundry support has been forgotten.

Roles: Caregiver / Coordinator

Caregiver: Mrs. Vale says nobody handled her laundry yesterday. Today's assignment lists breakfast and light tidying, not laundry.

Coordinator: Yesterday's note says the washing was started, but it doesn't confirm whether the clothes were put away.

Caregiver: So we have a partial record, not evidence that the whole task was completed or missed.

Coordinator: That's right. Could you ask what she wants help with now without suggesting someone failed her?

Caregiver: She says there are clean clothes in the basket, and she would like them folded before visitors arrive.

Coordinator: That fits the light household support in today's plan, provided the other agreed needs can still be met.

Caregiver: We have forty minutes left, and breakfast is finished. I'll ask whether folding is her priority for that time.

Coordinator: Please also clarify that we are dealing with today's preference, not deciding what happened during yesterday's visit.

Caregiver: Understood. I'll describe the basket as I find it and avoid attributing the unfinished task to anyone.

Coordinator: If time runs short, tell her which agreed task remains and ask me about arranging follow-up support.

Caregiver: I'll confirm the priority with her now, then report the actual outcome before the visit closes.

Coordinator: Thank you. That gives us a clear account of the present need without turning an incomplete note into blame.

Notice the professional language

'Doesn't confirm whether' distinguishes missing documentation from proof that something did not happen.

'Provided' introduces a condition; it does not promise that every requested task will fit.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 02 / 12 SPEAKING TURNS

A preference that changes mid-visit

A client and caregiver agree what to do when a previously accepted household task becomes unwelcome.

Roles: Client / Caregiver

Client: I know I asked you to arrange the side table, but now I'd rather leave everything where it is.

Caregiver: That's fine. I'll stop moving things. Would you like the two items I've already moved put back?

Client: Please put the photograph back, but the lamp can stay there. It gives me better light for reading.

Caregiver: The photograph returns to its original place, and the lamp stays in the new position. Is that right?

Client: Yes. My son suggested clearing the table completely, but I don't like how empty that makes it feel.

Caregiver: We don't have to follow that suggestion just because it was discussed earlier. Your current preference matters here.

Client: Will you have to tell him that I changed my mind about it? He may be disappointed.

Caregiver: I won't assume you want personal details shared. We can check the agreed communication arrangement with the coordinator.

Client: I'd prefer to explain it to him myself when he calls. Can we use the remaining time for the laundry?

Caregiver: We have twenty minutes, and folding the clean towels is within today's assignment. Would that be useful now?

Client: Yes, let's do those. Leave the clothes in the basket because I want to sort them later myself.

Caregiver: I'll fold the towels only, leave the clothes untouched, and check with you before making any further changes.

Notice the professional language

'Would you like ... put back?' offers repair without demanding a reason for the changed preference.

'Within today's assignment' marks an agreed limit while still offering useful support.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 03 / 12 SPEAKING TURNS

Correcting a report before it spreads

A caregiver contacts a supervisor after realizing a previous message overstated what was known.

Roles: Caregiver / Supervisor

Caregiver: I need to correct the message I sent at ten. I wrote that the client had refused breakfast, but that was inaccurate.

Supervisor: Tell me what actually happened and which part of the earlier statement needs to be corrected in the record.

Caregiver: The client said she did not want toast at that time. She did not say she wanted no breakfast at all.

Supervisor: Did you ask whether she wanted another option that was available within the agreed plan for this visit?

Caregiver: Yes. She chose the other planned option, and I provided that support. My earlier wording left out her choice.

Supervisor: Then the correction should distinguish declining one item from declining the whole meal. Don't silently replace the original entry.

Caregiver: I'll use the approved correction process, identify the earlier message, and state the client's words and the later choice.

Supervisor: Good. Has anyone else already received the inaccurate version, or is it still only in the internal visit record?

Caregiver: The incoming caregiver received my initial message, so I also need to make sure they receive the correction explicitly.

Supervisor: Please contact them through the approved channel and ask them to confirm they understand the updated account of breakfast.

Caregiver: I'll do that now. I won't interpret the change as a problem with appetite because the facts don't establish that.

Supervisor: Thank you for catching it. An accurate correction protects the client from decisions based on a broader claim than the evidence supports.

Notice the professional language

approved channel - An employer-authorized method for communicating particular workplace information.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 04 / 12 SPEAKING TURNS

Helpful without stepping outside the assignment

A caregiver and client discuss why a new request needs clarification before the caregiver can act.

Roles: Client / Caregiver

Client: The last person helped me with this, so I assumed you could do exactly the same thing during your visit.

Caregiver: I understand that expectation. Different workers may have different qualifications and assignments, so I need to check my own authorized duties.

Client: It feels frustrating to explain everything again whenever someone new arrives. I thought the plan would make this straightforward.

Caregiver: The plan should help us coordinate. My current version lists reminders, but it doesn't authorize me to make medication decisions or administer medicines.

Client: Are you saying the other worker did something wrong when they helped me last week with a different part of it?

Caregiver: No. I don't know their qualifications or exactly what task they performed, so I can't make that judgment from this conversation.

Client: Then what can you do now that will actually help me get an answer before the visit is over?

Caregiver: I can help contact the person named in the plan, explain the question accurately, and make clear which part needs their guidance.

Client: Please tell them I'm asking about the current instructions, not asking you to change anything on your own without approval.

Caregiver: I'll say that. I will also explain that we need a response through the approved route rather than relying on someone else's memory.

Client: All right. While we wait for that response, let's carry on with the other support that is already agreed for today.

Caregiver: Yes. We can continue those tasks, and I will update you on the contact attempt without pretending that an unanswered message is approval.

Notice the professional language

'I don't know their qualifications' avoids criticizing another worker on incomplete information.

'An unanswered message is not approval' distinguishes requesting guidance from receiving authorization.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 05 / 12 SPEAKING TURNS

A disagreement about the meal record

A caregiver and supervisor discuss a family concern about how a client's breakfast was described.

Roles: Supervisor / Caregiver

Supervisor: The family read 'poor breakfast' in your note and understood it to mean the food you offered was unsuitable or badly prepared.

Caregiver: That wasn't what I meant. I was trying to describe the amount remaining, but the wording was vague and sounded like a judgment.

Supervisor: What concrete information can you provide instead, without deciding why the client left some of the meal or assessing its nutritional adequacy?

Caregiver: I can state which approved option was served, that approximately half remained, and that the client said she had finished eating.

Supervisor: Did she ask for a different option, report a problem, or say anything that would support another explanation for what happened?

Caregiver: She asked whether a different approved breakfast could be offered tomorrow. She did not connect that request to a problem with today's food.

Supervisor: Then keep those facts separate. A request for variety is useful information, but it does not prove a clinical concern or dissatisfaction.

Caregiver: I'll use the approved correction process and explain that 'poor' was my unclear wording, not a quotation from the client herself.

Supervisor: Please also tell the family, through the authorized communication route, that the record is being clarified rather than making promises about future meals.

Caregiver: I can say the preference has been passed on and the future arrangement still needs confirmation within the existing care plan.

Supervisor: Exactly. We need a reliable account of the visit and a clear distinction between a request, an approved change, and an observation.

Caregiver: I'll correct the account now and check that the incoming caregiver receives the same clarified information before the next scheduled visit.

Notice the professional language

care plan - The current agreed record of a client's needs, support, and authorized arrangements.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 06 / 12 SPEAKING TURNS

An authorized contact is not an unlimited audience

A caregiver asks a coordinator about a request to broaden routine family updates.

Roles: Caregiver / Coordinator

Caregiver: The client's son is an authorized contact, but today he asked me to send the same update to three other relatives directly.

Coordinator: Have those additional recipients been checked against the client's sharing preferences and the approved contact arrangements for the information involved?

Caregiver: Not that I can establish. He said they all know about the support already, but I haven't treated that as permission to contact them.

Coordinator: Good. Familiarity with the situation does not tell us whether a particular disclosure to a particular person is authorized under our process.

Caregiver: He also suggested I remove the client's name from the update. The recipients would still know whose visit I was describing.

Coordinator: Then removing the name would not resolve the issue. We need to clarify the recipients, purpose, information, and approved communication route.

Caregiver: I told him I could pass on the request but could not promise that I would send future updates to the expanded group.

Coordinator: That was clear. Please let me handle the request through the established process and check the client's preferences before any change is made.

Caregiver: Should I continue the existing authorized update in the meantime, with only the details appropriate to that agreed purpose and recipient?

Coordinator: Yes, within the current arrangement. Do not broaden the content or audience simply because a new request is waiting to be reviewed.

Caregiver: I'll confirm that the request is pending and keep the current arrangement separate from any possible future change to the sharing permissions.

Coordinator: Thank you. That gives the family a clear next step while avoiding an accidental change to who receives the client's private information.

Notice the professional language

authorized contact - A person verified as permitted to receive specified information or take part in specified communication.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 07 / 12 SPEAKING TURNS

A handoff after a last-minute change

Two caregivers reconcile a changed client preference without erasing an earlier accurate note.

Roles: Incoming caregiver / Outgoing caregiver

Incoming caregiver: Your first note says the client wanted help arranging the books this afternoon, but a later message says to leave them alone.

Outgoing caregiver: Both reflect what the client said at different times. At nine she requested help; at ten thirty she changed her preference.

Incoming caregiver: Then the later message is a change in the request, not necessarily a correction of something you recorded wrongly at nine.

Outgoing caregiver: Exactly. The current preference is to leave the books where they are until she discusses it with you during your visit.

Incoming caregiver: Does that mean I should remove the task from every future visit, or only pause it for this afternoon's discussion?

Outgoing caregiver: Only the current request is clear. She hasn't asked for a permanent change, and I haven't represented it as one in the record.

Incoming caregiver: I'll check her preference directly before moving anything. Is another agreed task ready to take priority if she still wants the books left?

Outgoing caregiver: She mentioned folding clean towels, which fits the existing assignment. Please confirm that choice rather than assume it is still what she wants.

Incoming caregiver: Let me read it back: no book rearranging before checking, no assumed permanent change, and towels are a possible preference to confirm.

Outgoing caregiver: That's right. The key information is the timing of the change and the difference between a current preference and a revised care plan.

Incoming caregiver: I'll acknowledge the update in the approved system so you know I received it, and I'll record what she chooses at my visit.

Outgoing caregiver: Thank you. That closes this handoff while leaving room for the client to make her own decision again when you arrive.

Notice the professional language

care plan - The current agreed record of a client's needs, support, and authorized arrangements.

Rehearse this dialogue with an AI partner

FULL DIALOGUE 08 / 12 SPEAKING TURNS

Explaining a service problem without blaming the client

A coordinator and caregiver reconstruct an access delay to give the client an accurate response.

Roles: Coordinator / Caregiver

Coordinator: The client says we marked the visit as missed because she wasn't home, but she says she was waiting inside the whole time.

Caregiver: My actual observation was that the approved entry method failed and I received no response through the normal contact route at nine.

Coordinator: So the record should describe no access and no response, not establish that the client was away from the home at that time.

Caregiver: Correct. I cannot know where she was from that information alone. I need to correct the broader conclusion in the visit record.

Coordinator: Which steps did you take after that, and can you provide the times without adding a reason that nobody verified during the incident?

Caregiver: I contacted the coordinator on duty through the approved procedure at nine five and followed the instructions I received for the access problem.

Coordinator: The response to the client should acknowledge the disruption and clarify the inaccurate wording without claiming the entry issue was her fault.

Caregiver: I can explain that the record overstated what we knew, apologize for that wording, and describe the actual sequence through the authorized channel.

Coordinator: Good. Don't promise a particular complaint outcome or a future service change that has not been reviewed and approved through our process.

Caregiver: I'll confirm that the concern has been passed for review and identify how she will receive the next update about the access arrangement.

Coordinator: Please also ensure the next caregiver sees the verified entry instructions, not an informal suggestion from a neighbor or an assumption from this incident.

Caregiver: I'll use the approved record and request acknowledgment, so both the corrected account and the current authorized instructions reach the next person clearly.

Notice the professional language

'No response' is an observation; 'not home' is an inference that may be wrong.

A useful repair acknowledges the specific inaccurate claim and explains how it will be corrected.

Rehearse this dialogue with an AI partner

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Confirming the visit and the care plan

SHARED CASE

You arrive at 9:00 for a two-hour visit. Your assignment lists breakfast and light household tasks. The client expects transport to a 10:00 appointment, but transport is not listed and you have no authorization. The coordinator is available by phone.

Role A | Responding professional

Name the mismatch between the assignment and the client's expectation without blaming either person. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the client. The appointment is at 10:00, and you remember discussing transport last week, but you do not know whether it was booked.

Useful expressions

My assignment lists __; were you expecting something else?

Could we check whether __ was arranged separately?

I can confirm __, but I still need to check __.

Cover this until after your first attempt

Your appointment is at ten, and I understand why you expected a ride. My assignment today lists breakfast and light household tasks, but it does not include transport. May I contact the coordinator now to check what was arranged? I cannot confirm a ride until we have that information.

Second round

The coordinator says a separate transport service may have been contacted but needs five minutes to confirm. Explain the uncertainty and agree what to do while waiting.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Consent, choice, and respectful assistance

SHARED CASE

The client declines help arranging clothes before a visitor arrives. The visit has thirty minutes left. Choosing clothes is part of the agreed support, but the client prefers ten quiet minutes first. There is no immediate safety concern.

Role A | Responding professional

Make consent and preferences explicit while explaining a real time limit. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the client. You want ten minutes of quiet before deciding whether you want help. You are comfortable leaving the clothes until later.

Useful expressions

Would you prefer ___ or ___?

You can say no; I need to explain ___ so you can choose.

I'll give you ___, then check whether you would like ___.

Cover this until after your first attempt

You would like some quiet time before choosing clothes. We have thirty minutes left in today's visit. Would you prefer ten minutes alone and then help with the clothes, or would you rather leave that task today? Either way, I will respect your choice and confirm what support you want next.

Second round

The client chooses to leave the task today. Accept the decision and agree which remaining support, if any, they want.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Reporting a change without diagnosing

SHARED CASE

During a fictional routine visit, the client says, 'I feel more tired than yesterday.' You notice they stop twice while describing breakfast, which is unusual for your visits. You are not qualified to diagnose. No emergency signs are specified; use the local procedure for reporting changes and urgent concerns.

Role A | Responding professional

Report a change using a source, time, and clear limit on your knowledge. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the supervisor. Ask when the change was noticed, what the client actually said, and what the caregiver observed. You will use the organization's approved escalation process.

Useful expressions

At ___, I noticed ___; the client said ___.

Compared with my usual visits, ___.

I cannot determine the cause; I am reporting ___.

Cover this until after your first attempt

The client told me, 'I feel more tired than yesterday.' During our conversation, they paused twice before finishing their account of breakfast. That differs from my usual visits, but I cannot identify the cause. I am reporting the change through our agreed process and need the authorized clinician's direction on the next step.

Second round

The supervisor asks whether the client is 'just tired.' Restate the facts and your knowledge limit without agreeing to an unsupported explanation.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Boundaries around medicines and clinical questions

SHARED CASE

Your fictional assignment permits reminders only, not administering medicines or recommending doses. The client asks whether to double a missed dose. You do not know the medicine details. The care plan identifies a nurse contact for medication questions.

Role A | Responding professional

Respond helpfully to a request that exceeds the caregiver's authorized role. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the client. You want a quick answer about a missed dose and are worried that a call will take too long. The nurse contact is in your care plan.

Useful expressions

I can help with ___, but I am not authorized to ___.

That question needs ___; may we contact them?

I don't want to guess about ___. Let's check the approved route.

Cover this until after your first attempt

I understand you want to know what to do about the missed dose. My role permits reminders, but I cannot recommend a dose or tell you to double it. Let's contact the nurse using the number in your care plan. I can help communicate your question without making the medication decision myself.

Second round

The client says a neighbor gave an answer already. Explain why you still cannot endorse that advice and offer to help contact the appropriate professional.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

Meals, preferences, and unresolved restrictions

SHARED CASE

The plan lists two approved breakfast options. The client dislikes the first and asks for a third food that is not on the list. You do not know whether it meets the documented restrictions. The second approved option is available. The coordinator can clarify the third option.

Role A | Responding professional

Separate an available approved option from a requested but unverified substitution. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the client. You dislike the first listed breakfast option. You will accept the second if the requested third choice cannot yet be confirmed.

Useful expressions

The plan lists ____; the requested substitute still needs ____.

Is that a preference, or is it part of the documented restriction?

I can offer ____ now, or help check ____.

Cover this until after your first attempt

You don't want the first breakfast option today. The second option in your plan is available, or I can ask the coordinator about the food you requested. I don't know whether that substitution fits your documented restrictions, so I should not describe it as approved. Which of those next steps would you prefer?

Second round

The client asks why the restriction exists. You do not know the clinical reason; acknowledge the question and refer it without making one up.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

Privacy and family communication

SHARED CASE

An unfamiliar caller says she is the client's niece and asks what happened during the visit. The contact record is unavailable to you at that moment. You cannot verify her identity or permission. Your coordinator can handle the request through the approved process.

Role A | Responding professional

Explain an identity or permission check and provide a legitimate route for the caller's request. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the caller. You say you are the niece and know the client's address, but the caregiver cannot verify you. You can contact the agency directly.

Useful expressions

Before discussing ___, I need to verify ___.

I can pass on your request without sharing ___.

Please use ___ so the authorized person can check ___.

Cover this until after your first attempt

I understand you are concerned about your aunt. Before discussing visit details, I need to verify the contact arrangement through our approved process. I cannot confirm private information on this call. Please contact the coordinator through the agency's usual number, or I can pass on your request without sharing the client's details.

Second round

The caller says the check is insulting. Acknowledge the frustration and explain that the same process protects every client's privacy.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

A handoff the next person can use

SHARED CASE

At 11:00 you finish a fictional visit. Breakfast support and laundry are complete. A transport query for tomorrow's 10:00 appointment is still pending with the coordinator. The incoming caregiver starts at 15:00. The coordinator, not the incoming caregiver, is arranging transport.

Role A | Responding professional

Separate completed work, unresolved work, and responsibility for the next step. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the incoming caregiver. Your visit starts at 15:00, and you need to know whether tomorrow's transport is confirmed and who is arranging it.

Useful expressions

Completed: ___. Still pending: ___.

___ is handling the follow-up; please check ___ by ___.

Could you read back the status and the person responsible?

Cover this until after your first attempt

Breakfast support and laundry are complete. Transport for tomorrow's ten o'clock appointment is still unconfirmed; the coordinator owns that follow-up. Your visit starts at three today. Please check the coordinator's update through the approved channel, and do not describe the ride as booked until confirmation arrives. Could you read back the pending item and who is handling it?

Second round

The incoming caregiver assumes that checking the update means personally booking another ride. Correct the misunderstanding before a duplicate request is made.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Delays, complaints, and reliable follow-up

SHARED CASE

Your fictional 9:00 visit will be delayed. At 8:50, dispatch estimates arrival between 9:30 and 9:45 but cannot guarantee it. The client had planned breakfast support at 9:00. Dispatch is checking an approved backup arrangement and will update the client by 9:10.

Role A | Responding professional

Acknowledge the disruption, label an estimate, and commit only to an update that has been agreed. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are the client. You expected breakfast support at 9:00 and want a reliable next step, not another vague apology. You understand an estimate is not a guarantee.

Useful expressions

I'm sorry this affects _____. What we know now is _____.

The current estimate is _____; it is not yet guaranteed.

_____ will update you by _____ about _____.

Cover this until after your first attempt

I'm sorry this delay affects your planned breakfast support. Dispatch currently estimates arrival between nine thirty and nine forty-five, but that is not a guaranteed time. They are checking the approved backup arrangement and will update you by nine ten. I will not describe the backup as confirmed until dispatch verifies it.

Second round

At 9:10 no update has arrived. Use the approved follow-up route and describe the missed update accurately without assuming a backup was cancelled.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

Lesson conversations and speaking practice

The following sections match 05 and 06 on the industry webpage and in the learner workbook. Each lesson has three ten-turn conversations and two bounded speaking scenarios. These are original fictional teaching scripts, not transcripts of real people.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

The appointment is not on my assignment

A caregiver arrives for the fictional 9:00 visit and checks an unexpected transport request.

01 • Client: You're here. We need to leave for my clinic appointment in forty minutes.

02 • Caregiver: Your appointment is at ten, correct? I see breakfast and light household tasks on my assignment.

03 • Client: I mentioned the ride when I spoke with someone last week.

04 • Caregiver: You may have discussed a separate arrangement. I don't see transport listed here.

05 • Client: Does that mean nobody is coming to pick me up today?

06 • Caregiver: I haven't confirmed that. May I call the coordinator before we make that assumption?

07 • Client: Yes, please. I don't want to miss another appointment this month.

08 • Caregiver: I'll explain the appointment time and ask what transport, if any, was arranged.

09 • Client: While you check, can I have my breakfast at the table?

10 • Caregiver: Certainly. We'll continue the agreed visit tasks while the coordinator checks the transport details.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

A changed start time

A caregiver and coordinator reconcile two different visit times before a planned home visit.

01 • Caregiver: My assigned visit starts at nine tomorrow, but the client's reminder says half past nine.

02 • Coordinator: I changed the visit window yesterday. Your printed schedule may be an older version.

03 • Caregiver: Which time should I confirm with the client before I leave today?

04 • Coordinator: Half past nine. The visit is still two hours, ending at eleven thirty.

05 • Caregiver: The client usually expects breakfast earlier. Has that change been discussed with her?

06 • Coordinator: Not yet. I need to check whether the later time works for her.

07 • Caregiver: Then I shouldn't present nine thirty as an agreed change at this point.

08 • Coordinator: Correct. Please tell her I'm checking the revised arrangement and will call shortly.

09 • Caregiver: I'll distinguish the proposed time from the confirmed plan and wait for your update.

10 • Coordinator: Thank you. I'll confirm the final window with both of you before today's visit ends.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

Whose preference is it?

A client explains how they want ordinary household support organized during the visit.

01 • Caregiver: Your plan includes light housekeeping today: the dishes, laundry, and clearing the table. Which would you prefer first?

02 • Client: My daughter always says to start with the laundry, but I'd rather clear the table.

03 • Caregiver: You'd like the table ready before we deal with the laundry. Have I understood?

04 • Client: Exactly. I enjoy sitting there, and the folded towels aren't urgent today.

05 • Caregiver: That order fits today's assignment. Shall we keep your newspapers in their usual basket?

06 • Client: Please leave the top one out. I'm halfway through an article in it.

07 • Caregiver: I'll leave that newspaper on the side table and put the others in the basket.

08 • Client: Thank you for asking. Sometimes things disappear before I've finished using them.

09 • Caregiver: I'll check before moving personal items. Let me know if you'd prefer another arrangement.

10 • Client: That works. Once the table is clear, we can decide which laundry needs attention.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Confirming the visit and the care plan

You arrive at 9:00 for a two-hour visit. Your assignment lists breakfast and light household tasks. The client expects transport to a 10:00 appointment, but transport is not listed and you have no authorization. The coordinator is available by phone.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the client. The appointment is at 10:00, and you remember discussing transport last week, but you do not know whether it was booked.

Second round: The coordinator says a separate transport service may have been contacted but needs five minutes to confirm. Explain the uncertainty and agree what to do while waiting.

Scenario 2 | A request from a visiting relative

During a two-hour visit, a relative asks you to clean the garage. Your assignment covers breakfast and light tasks in the living area. The client wants the remaining time used for laundry. No garage work is authorized.

Role A: Caregiver: acknowledge the relative, explain the assigned duties, and check the client's preference.

Role B: Relative: you thought any household cleaning was included. You can contact the coordinator about future services.

Possible opening: Could you clean out the garage before you leave today?

Success checks

- Identify the actual assignment without judging the relative.
- Keep the client's stated preference in the discussion.
- Refer any new service request to the coordinator without accepting a private arrangement.

Add a complication: The relative offers extra cash. Your employer requires service changes to go through the coordinator, not private arrangements.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Ten minutes before we start

A client requests quiet time during a scheduled visit with thirty minutes remaining.

01 • Client: I'd rather not sort my clothes right now. I've had people coming in all morning.

02 • Caregiver: You'd like a break from activity. Would ten quiet minutes help before we discuss it again?

03 • Client: Yes, but I don't want you standing beside the chair while I rest.

04 • Caregiver: Of course. I can wait in the kitchen if that suits you and the visit arrangement.

05 • Client: That's fine. Please don't start opening drawers while I'm having my break.

06 • Caregiver: I won't. Your personal space matters. We have thirty minutes left, so we can choose a smaller task afterward.

07 • Client: And if I decide not to do the clothes at all today?

08 • Caregiver: I'll respect that choice. We can confirm what other agreed support you want instead.

09 • Client: Good. Please come back in ten minutes and ask me then.

10 • Caregiver: Ten minutes in the kitchen, then I'll check with you before we do anything else.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

Let me finish the sentence

A caregiver repairs an overly hurried offer of help with a household task.

01 • Caregiver: Shall I put those books away for you before your visitor arrives this afternoon?

02 • Client: I was going to say that I want the green one left out.

03 • Caregiver: I'm sorry; I started moving them before you finished. I'll stop and listen.

04 • Client: The green book stays here, and the other two belong on the lower shelf.

05 • Caregiver: Green book here, two books on the lower shelf. Is that the arrangement you want?

06 • Client: Yes. I know it seems small, but I like to find things myself.

07 • Caregiver: That makes sense. Would you like me to move those two now, or leave them?

08 • Client: You can move them now. Just keep them where I can see their covers.

09 • Caregiver: I'll place them with their covers visible and ask you to check the position.

10 • Client: Thank you. That's the help I wanted, without everything being reorganized around me.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

The family member answers first

A caregiver makes room for the client's own preference during a relative's visit.

01 • Relative: She wants you to rearrange the whole cupboard today. We discussed it yesterday evening.

02 • Caregiver: Thank you. I'd like to check her preference directly before we agree on that task.

03 • Relative: She's tired, so I'm trying to save her from having to explain everything.

04 • Caregiver: I appreciate that. We can keep the question short and give her time to answer.

05 • Relative: What if she changes her mind after we have already started moving things?

06 • Caregiver: Then we'll pause and ask what she wants to keep or change before continuing.

07 • Relative: All right. She may only want the cups moved onto the lower shelf.

08 • Caregiver: That would be a more specific request. I'll ask her which items she wants within reach.

09 • Relative: I'll let her answer, and I can explain yesterday's discussion if she asks me.

10 • Caregiver: Thank you. We can support the conversation without replacing her own choice with our assumptions.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Consent, choice, and respectful assistance

The client declines help arranging clothes before a visitor arrives. The visit has thirty minutes left. Choosing clothes is part of the agreed support, but the client prefers ten quiet minutes first. There is no immediate safety concern.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the client. You want ten minutes of quiet before deciding whether you want help. You are comfortable leaving the clothes until later.

Second round: The client chooses to leave the task today. Accept the decision and agree which remaining support, if any, they want.

Scenario 2 | An unwanted photograph

A relative wants to photograph the caregiver helping the client choose clothes. The client says they do not want a photograph. Your employer requires consent and approval for work-related images. No image has been taken.

Role A: Caregiver: support the client's refusal and refer questions about photographs to the coordinator.

Role B: Relative: you wanted a family update, not a public post. You can describe the visit without taking a photograph.

Possible opening: Can you both smile for a quick photo to send the family?

Success checks

- Acknowledge the relative's purpose without treating it as consent.
- Restate that the client does not agree to a photograph.
- Offer an appropriate non-photographic way to give a general update.

Add a complication: The relative says the image is only for a private group. The client's refusal has not changed.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

What did you actually notice?

A caregiver gives a factual phone report to a supervisor using the fictional case details.

01 • Supervisor: Your message says the client seems different today. What exactly have you noticed during the visit?

02 • Caregiver: She said, 'I feel more tired than yesterday,' and paused twice while describing breakfast to me.

03 • Supervisor: How does that compare with her usual baseline during your morning visits? Are those pauses typical?

04 • Caregiver: Not during my usual visits, but I don't know whether someone else has noticed them before.

05 • Supervisor: When did this conversation happen, and have you recorded the client's own words accurately?

06 • Caregiver: It happened at nine fifteen. Those are her words, and I haven't added a possible diagnosis.

07 • Supervisor: Thank you. We need the authorized clinician's direction through our agreed reporting process.

08 • Caregiver: Understood. I can provide the time and observations, but I cannot determine what caused the change.

09 • Supervisor: Stay within the local procedure, and use the urgent escalation route if an urgent concern arises.

10 • Caregiver: I'll follow that procedure and confirm who receives this report rather than assuming the message is enough.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

A quieter morning

A caregiver checks the meaning of a client's unusual silence without attributing a cause.

01 • Caregiver: You've been quieter than during my recent visits. Would you like some quiet, or is something different today?

02 • Client: I'm thinking about a family matter. I don't feel like having much conversation this morning.

03 • Caregiver: Thank you for telling me. Would you prefer that I keep questions limited to the support you need?

04 • Client: Yes, please. I still want breakfast at the table, just without the usual chat today.

05 • Caregiver: We can do that. Let me know if there is any change or anything you want me to report.

06 • Client: I will. Please don't tell the coordinator that I'm upset with you, because I'm not.

07 • Caregiver: I won't assume that. I'll describe your stated preference rather than give a reason you haven't given.

08 • Client: That's helpful. People sometimes decide what my silence means before they ask me about it.

09 • Caregiver: I'll check instead of guessing. Is there anything else about today's visit you'd like adjusted?

10 • Client: No, the usual support is fine. A quieter visit is all I'm asking for right now.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A secondhand description

A coordinator helps a caregiver distinguish a relative's comment from a direct observation.

01 • Caregiver: The client's brother said she seemed unsteady yesterday, but I wasn't present during his visit.

02 • Coordinator: Then identify him as the source. Have you noticed any change yourself during today's visit?

03 • Caregiver: I haven't observed the situation he described. That doesn't establish that it didn't happen yesterday.

04 • Coordinator: Exactly. What words did he use, and did he say when he noticed it?

05 • Caregiver: He said, 'She seemed unsteady after lunch.' He didn't give a more specific time.

06 • Coordinator: Please keep that distinction clear when you pass the concern through the agreed reporting route.

07 • Caregiver: I'll say it is his report, that the timing is approximate, and that I did not witness it.

08 • Coordinator: Good. Don't label the earlier report as false simply because your own observation differs today.

09 • Caregiver: Understood. I'll avoid combining the two accounts into a single conclusion about her condition.

10 • Coordinator: That will help the authorized person assess what information is available and what still needs clarification.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Reporting a change without diagnosing

During a fictional routine visit, the client says, 'I feel more tired than yesterday.' You notice they stop twice while describing breakfast, which is unusual for your visits. You are not qualified to diagnose. No emergency signs are specified; use the local procedure for reporting changes and urgent concerns.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the supervisor. Ask when the change was noticed, what the client actually said, and what the caregiver observed. You will use the organization's approved escalation process.

Second round: The supervisor asks whether the client is 'just tired.' Restate the facts and your knowledge limit without agreeing to an unsupported explanation.

Scenario 2 | An incomplete overnight report

A relative says the client 'wasn't herself last night' but gives no specific behavior. You have just arrived and have not yet observed a change. The supervisor is reachable. No emergency signs are supplied.

Role A: Caregiver: ask for observable details and timing, identify the source, and follow the agreed reporting process.

Role B: Relative: clarify that the client spoke less than usual at dinner at 7:00. You do not know why.

Possible opening: She wasn't herself last night. Can you tell the nurse something is wrong?

Success checks

- Ask what was seen or heard and when.
- Separate the relative's report from your own observations.
- Decline to diagnose and identify the appropriate reporting route.

Add a complication: The relative asks you to name a likely condition. You are not qualified to diagnose.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

Please tell me which dose

The caregiver responds to the fictional missed-dose request without offering medical advice.

01 • Client: I missed yesterday's medicine. Should I take twice as much today to make up for it?

02 • Caregiver: I understand why you're asking, but I cannot recommend a dose or tell you to double it.

03 • Client: You remind me about medicines, so I thought you would know what to do.

04 • Caregiver: A medication reminder is part of my assignment. Choosing a dose is outside my scope of duties.

05 • Client: I don't want to spend the whole morning trying to reach the right person.

06 • Caregiver: The care plan gives a nurse contact for these questions. May I help you contact that person?

07 • Client: Yes. What should I say when we reach the nurse on the phone?

08 • Caregiver: Explain that you missed a dose and ask for their guidance. I can help communicate the question.

09 • Client: So you aren't saying the answer is yes or no to taking extra?

10 • Caregiver: Correct. I'm saying that decision needs authorized clinical advice, and I won't guess about it.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

A relative's new instruction

A caregiver discusses an unverified change to an authorized task with the coordinator.

01 • Caregiver: The client's daughter says my duties now include administering medicine, but my current assignment permits reminders only.

02 • Coordinator: Have you received an updated plan or confirmation from the authorized person responsible for that change?

03 • Caregiver: No. The only new information is the daughter's statement during today's visit at nine thirty.

04 • Coordinator: Then don't treat that statement alone as a change to your authorized duties. I'll check the plan.

05 • Caregiver: I can continue the agreed nonclinical tasks while you clarify, but I won't perform the additional task.

06 • Coordinator: That's the correct distinction. Tell the client we are checking the arrangement, not abandoning the request.

07 • Caregiver: Should I explain that the question is about authorization, not whether I trust the family member?

08 • Coordinator: Yes. Keep the language factual and avoid implying that she deliberately gave you incorrect information.

09 • Caregiver: I'll say the current assignment and the new request differ, and that we need an authorized clarification.

10 • Coordinator: I'll contact the responsible person now and confirm the outcome through the approved communication channel.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

The neighbor said it was fine

A client asks the caregiver to endorse advice received from a neighbor.

01 • Client: My neighbor takes something similar and says I can change the timing. Is that all right?

02 • Caregiver: I can't confirm that advice. Similar medicines or situations may still involve different instructions for different people.

03 • Client: But she has been taking it for years and sounded very sure when we spoke.

04 • Caregiver: I hear that she was confident. Her experience doesn't give me authority to advise on your medicine.

05 • Client: I just want someone to tell me whether her suggestion makes sense for me.

06 • Caregiver: That question belongs with an appropriately qualified professional. We can use the contact route in your plan.

07 • Client: Can you explain what she suggested when we call, so I don't forget the details?

08 • Caregiver: I can help relay your question, making clear that it came from a neighbor and is not an instruction.

09 • Client: All right. Please don't describe it as something I have already decided to do.

10 • Caregiver: I won't. I'll distinguish advice you heard from a decision or action you have actually taken.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Boundaries around medicines and clinical questions

Your fictional assignment permits reminders only, not administering medicines or recommending doses. The client asks whether to double a missed dose. You do not know the medicine details. The care plan identifies a nurse contact for medication questions.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the client. You want a quick answer about a missed dose and are worried that a call will take too long. The nurse contact is in your care plan.

Second round: The client says a neighbor gave an answer already. Explain why you still cannot endorse that advice and offer to help contact the appropriate professional.

Scenario 2 | An unreadable label

The client asks you to interpret a partly unreadable medicine label. Your assignment allows reminders only. The client has the pharmacy contact details, and your agency has an approved route for medication questions. No dosing decision has been made.

Role A: Caregiver: explain that you cannot fill in missing instructions from memory and help use the approved contact route.

Role B: Client: you believe you remember the wording but are not certain. You agree to ask an authorized professional.

Possible opening: The print has rubbed off. Can you tell me what this instruction probably says?

Success checks

- Identify the information as unreadable or unverified.
- Do not interpret or invent the missing instruction.
- Offer a concrete route for obtaining authorized clarification.

Add a complication: The client finds an old label from a previous supply and asks whether it must be the same.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

A third breakfast option

Fictional care plan: today's listed options are porridge or toast. Pancakes have not been reviewed as a substitute; this exchange does not assess dietary suitability.

01 • Client: Could you make the breakfast I had at my daughter's house? I'd like something different from porridge.

02 • Caregiver: What did you have there? I need the name of the dish before I check today's plan.

03 • Client: Pancakes. She served them plain, and I'd like those instead of porridge this morning.

04 • Caregiver: Today's plan lists porridge or toast, but not pancakes. I'll ask the coordinator about that request.

05 • Client: Could I have toast now, then ask about pancakes for a later visit? I don't want to wait.

06 • Caregiver: Yes, toast is the other listed choice. I'll prepare that and pass on your request for more variety.

07 • Client: Please don't put down that I refused breakfast. I'm choosing the toast you've offered.

08 • Caregiver: I'll record that choice accurately. The pancake request is a separate question for the coordinator.

09 • Client: Could the coordinator also explain why those are the only two options on today's plan?

10 • Caregiver: I'll ask. Their absence from this list doesn't tell me why, so I won't invent a reason.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

Half a portion is not no breakfast

Two caregivers clarify the meaning of a meal record during a handoff.

01 • Incoming caregiver: The note says breakfast was incomplete. Does that mean the client didn't eat anything this morning?

02 • Outgoing caregiver: No. I observed about half the portion remaining after the client said she was finished.

03 • Incoming caregiver: Was that weighed, or is half your visual estimate of what remained on the plate?

04 • Outgoing caregiver: It's an estimate, not a measured amount. I should make that clear in the intake record.

05 • Incoming caregiver: Did she give a reason for leaving it, or would we be guessing about that?

06 • Outgoing caregiver: She said she had finished. She did not give a reason, and I haven't supplied one.

07 • Incoming caregiver: Then I should not repeat the information as a refused meal or a confirmed appetite problem.

08 • Outgoing caregiver: Correct. Please preserve the distinction between what was served, what remained, and what she actually said.

09 • Incoming caregiver: I'll use that wording and follow the plan's reporting process if there is a concern.

10 • Outgoing caregiver: Thank you. I'll clarify the original note so the same misunderstanding doesn't continue into another handoff.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

A shopping substitution needs checking

A caregiver checks a proposed grocery substitute with a coordinator rather than interpreting dietary instructions.

01 • Caregiver: The listed product wasn't available in the shop, and the client has asked about another brand.

02 • Coordinator: Are you asking about a brand preference or whether the substitute meets a documented dietary restriction?

03 • Caregiver: The second issue. I can identify the product, but I am not qualified to interpret its clinical suitability.

04 • Coordinator: Then don't tell the client it is equivalent just because the packaging looks similar to the usual one.

05 • Caregiver: I haven't. The original approved alternative is still available at home for today's planned meal.

06 • Coordinator: Good. Use the agreed contact route to clarify the proposed substitute before making any claim about it.

07 • Caregiver: I'll explain that the choice is pending, not rejected, and that an existing approved option is available.

08 • Coordinator: Please also record what was unavailable without changing the shopping list as though the substitute were approved.

09 • Caregiver: Understood. I'll keep the requested change separate from the current instruction and confirm the response when it arrives.

10 • Coordinator: That will let the appropriate person decide with the actual product information instead of our assumptions.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Meals, preferences, and unresolved restrictions

The plan lists two approved breakfast options. The client dislikes the first and asks for a third food that is not on the list. You do not know whether it meets the documented restrictions. The second approved option is available. The coordinator can clarify the third option.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the client. You dislike the first listed breakfast option. You will accept the second if the requested third choice cannot yet be confirmed.

Second round: The client asks why the restriction exists. You do not know the clinical reason; acknowledge the question and refer it without making one up.

Scenario 2 | The relative's favorite recipe

A relative brings ingredients for a meal that is not on the client's approved list. The client would like to try it. You cannot confirm whether it fits a documented restriction. A listed alternative is available now.

Role A: Caregiver: distinguish a welcome offer from an approved meal and propose an authorized check.

Role B: Relative: you want to help and did not know there was a restriction. You can provide the ingredient information.

Possible opening: I brought everything for his favorite meal. Can you prepare it today?

Success checks

- Acknowledge the offer without endorsing the proposed meal.
- Explain exactly what remains unverified.
- Offer the existing approved choice and the appropriate clarification route.

Add a complication: The relative says removing one ingredient should solve the issue. No authorized person has confirmed that.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

An unfamiliar caller

A caregiver handles a request for private visit information from an unverified person.

01 • Caller: I'm Mrs. Vale's niece. Tell me whether she ate breakfast and what happened during your visit.

02 • Caregiver: I understand you're asking about her welfare. Before discussing visit details, I need to verify the contact arrangement.

03 • Caller: I can give you her address and her birthday. Surely that shows who I am.

04 • Caregiver: I still need to follow the approved verification process. I can't confirm private details on this call.

05 • Caller: That sounds as though you think I am lying about being part of the family.

06 • Caregiver: I'm not making that judgment. We use the same privacy process for requests involving every client.

07 • Caller: Then how can I get an answer without explaining the whole situation to several people?

08 • Caregiver: Please contact the coordinator through the agency's usual number, or I can pass on your request.

09 • Caller: You can pass on the request, but please make clear that I would like a call back.

10 • Caregiver: I'll relay that request through the approved route without sharing the client's visit details here.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

The conversation in the hallway

A caregiver moves a potentially private discussion away from a shared building corridor.

01 • Relative: Before you leave, tell me what the nurse said yesterday. The neighbors won't mind us talking here.

02 • Caregiver: I'd prefer not to discuss private care information in a shared hallway where other people may hear.

03 • Relative: I'm an authorized contact, so I thought it would be all right to ask you now.

04 • Caregiver: Your authorization still needs to match the information being requested, and the setting also matters for privacy.

05 • Relative: Can we step inside, then? I don't want to hold you up at the end of your visit.

06 • Caregiver: We can use the agreed private contact arrangement. I also need to confirm which information I am authorized to share.

07 • Relative: I only want to know whether someone will call me about the question I raised.

08 • Caregiver: I can pass that request to the coordinator without discussing clinical details I haven't been authorized to communicate.

09 • Relative: All right. Please ask them to use the contact number they already have for me.

10 • Caregiver: I'll use the approved channel and make clear that you are requesting a response, not that one is already scheduled.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

A convenient but unapproved message

Two caregivers discuss how to share a handoff without using a personal messaging group.

01 • Incoming caregiver: The work system is slow. Can you send the visit details in our personal group chat instead?

02 • Outgoing caregiver: That group chat isn't approved for client information. We still need to protect confidentiality when the work system is slow.

03 • Incoming caregiver: We both work with this client, and I need the information before the next visit begins.

04 • Outgoing caregiver: I agree the handoff matters. Let's contact the coordinator for the approved backup method rather than invent one.

05 • Incoming caregiver: Would a photograph of the note be any different if I delete it from the group later?

06 • Outgoing caregiver: It would still share private information through that channel. Deleting it later wouldn't make the original choice approved.

07 • Incoming caregiver: Then I'll call the coordinator and explain the timing issue before my visit starts.

08 • Outgoing caregiver: I'll keep the handoff ready through the authorized route and confirm that you receive it.

09 • Incoming caregiver: Please flag any time-sensitive concern using the established escalation process while we resolve the system problem.

10 • Outgoing caregiver: I will. We need both a timely transfer and an appropriate channel, not an assumed exception.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Privacy and family communication

An unfamiliar caller says she is the client's niece and asks what happened during the visit. The contact record is unavailable to you at that moment. You cannot verify her identity or permission. Your coordinator can handle the request through the approved process.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the caller. You say you are the niece and know the client's address, but the caregiver cannot verify you. You can contact the agency directly.

Second round: The caller says the check is insulting. Acknowledge the frustration and explain that the same process protects every client's privacy.

Scenario 2 | A family group invitation

A relative invites you into a family messaging group to send daily visit updates. The agency has not approved that group, and the client's sharing preferences have not been checked. The coordinator can explain approved communication options.

Role A: Caregiver: decline the unapproved channel and offer the established route for updates.

Role B: Relative: you want everyone to receive the same information. You can ask the coordinator to clarify authorized contacts.

Possible opening: I'll add you to our family chat so you can post a quick update after every visit.

Success checks

- Explain that removing a name does not establish permission or channel approval.
- Avoid sharing visit details while discussing the request.
- Direct the family to the coordinator for a verified arrangement.

Add a complication: The relative offers to remove the client's name from the messages, but the group members would still know who the updates concern.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Requested is not booked

Outgoing and incoming caregivers confirm the status of tomorrow's transport.

01 • Outgoing caregiver: Breakfast support and laundry are complete. The unresolved item is transport for tomorrow's ten o'clock appointment.

02 • Incoming caregiver: So the ride has been requested, and I should tell the client it is booked?

03 • Outgoing caregiver: Please don't say booked yet. The coordinator is checking the arrangement, and confirmation has not arrived.

04 • Incoming caregiver: Do you want me to arrange another ride when I arrive for the three o'clock visit?

05 • Outgoing caregiver: No. The coordinator is the follow-up owner. Please check their update before arranging anything separately.

06 • Incoming caregiver: Let me read that back: transport is pending, and the coordinator is responsible for arranging it.

07 • Outgoing caregiver: Correct. Your task is to check the status and avoid presenting an unconfirmed request as a booking.

08 • Incoming caregiver: If there is still no update at three, I'll contact the coordinator using the established route.

09 • Outgoing caregiver: Thank you. The client knows it is being checked but has not been promised a pickup time.

10 • Incoming caregiver: I'll keep that wording consistent and confirm that I received the update when it becomes available.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

Which appointment did you mean?

A read-back exposes two different appointment dates during a routine handoff.

01 • Incoming caregiver: I heard you say the transport query concerns Thursday's appointment at ten. Is that the pending item?

02 • Outgoing caregiver: I said tomorrow at ten, but you're right that we should use the actual day and date.

03 • Incoming caregiver: There is also a Thursday appointment in the calendar, so 'the appointment' could mean either one.

04 • Outgoing caregiver: The query concerns Tuesday's ten o'clock appointment. Thursday's arrangement is a separate item that I haven't checked.

05 • Incoming caregiver: Then I should not carry your transport update over to Thursday simply because the destination is the same.

06 • Outgoing caregiver: Exactly. Please connect the message to Tuesday's appointment and leave Thursday's status unconfirmed unless separately verified.

07 • Incoming caregiver: I'll confirm the calendar entry through the approved record before I discuss it with the client.

08 • Outgoing caregiver: Good. I will also clarify my original message so another reader does not inherit the ambiguity.

09 • Incoming caregiver: Tuesday at ten, transport pending with the coordinator. Thursday's status is not established by this handoff.

10 • Outgoing caregiver: That's the distinction I needed. Thank you for checking the date rather than assuming we meant the same visit.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

An unread update

A caregiver checks whether a time-sensitive handoff has actually reached its recipient.

01 • Caregiver: I sent the handoff through the approved system, but I haven't received confirmation that it was read.

02 • Coordinator: Is there a time-sensitive item the incoming caregiver needs before the next visit starts today?

03 • Caregiver: Yes. The client asked that a specific household task be left until they can discuss it together.

04 • Coordinator: Then don't assume the system delivery notice means the caregiver understood the client's request.

05 • Caregiver: I'll use the approved backup contact route and ask for a read-back of that particular preference.

06 • Coordinator: Keep the message focused. They need the current preference, not every unrelated detail from the morning visit.

07 • Caregiver: I will say which task is on hold and that the client wants to discuss it before anything is moved.

08 • Coordinator: Please tell me if you cannot reach the incoming person through the agreed route before the visit.

09 • Caregiver: Understood. Sending the message is one step; confirming receipt and understanding is the remaining communication task.

10 • Coordinator: Exactly. Once that confirmation arrives, update the handoff status so nobody starts another unnecessary contact attempt.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | A handoff the next person can use

At 11:00 you finish a fictional visit. Breakfast support and laundry are complete. A transport query for tomorrow's 10:00 appointment is still pending with the coordinator. The incoming caregiver starts at 15:00. The coordinator, not the incoming caregiver, is arranging transport.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the incoming caregiver. Your visit starts at 15:00, and you need to know whether tomorrow's transport is confirmed and who is arranging it.

Second round: The incoming caregiver assumes that checking the update means personally booking another ride. Correct the misunderstanding before a duplicate request is made.

Scenario 2 | An action with two assumed owners

The outgoing caregiver thinks the coordinator ordered replacement household supplies. The coordinator thinks the incoming caregiver will ask the client what is needed first. No order has been placed. The incoming visit starts at 15:00.

Role A: Outgoing caregiver: clarify the actual status and obtain an explicit owner and next step.

Role B: Coordinator: confirm that you will arrange the approved order after the incoming caregiver checks the list with the client.

Possible opening: Before I hand over, can we confirm whether the supply order has actually been placed?

Success checks

- Keep an assumption separate from confirmed status.
- Name the person who checks the list and the person who places the order.
- Ask for a read-back of the sequence and timing.

Add a complication: A colleague says, 'It's probably already done.' No confirmation or order record is available.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A delay before breakfast support

A caregiver informs the client of a delay before the original visit window.

01 • Caregiver: I'm calling because today's nine o'clock visit will be delayed, and I know you planned breakfast support for that time.

02 • Client: How long will I be waiting? I need more than an apology before I can decide what to do.

03 • Caregiver: Dispatch currently estimates arrival between nine thirty and nine forty-five. That is an estimate, not a guaranteed arrival.

04 • Client: Is someone else definitely coming earlier instead, or are you still trying to arrange that?

05 • Caregiver: Dispatch is checking the contingency plan for delayed visits. They will tell you by nine ten what backup, if any, is confirmed.

06 • Client: Please don't tell me a backup is coming if you only know that someone is asking about it.

07 • Caregiver: I agree. I will distinguish a request from a confirmed arrangement and follow the agency's delay procedure.

08 • Client: Who should I contact if the promised update doesn't arrive by nine ten this morning?

09 • Caregiver: Use the agency's usual contact route for missed updates. I'll also make sure dispatch has your concern.

10 • Client: All right. I understand the current estimate and that the next confirmed commitment is the update, not the arrival.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

The update was missed

A caregiver follows up with dispatch after an agreed callback time passes.

01 • Caregiver: The client says the promised nine-ten update hasn't arrived. I'm following up through the approved delay process.

02 • Dispatcher: I haven't confirmed the backup yet. I was waiting for a response before calling the client.

03 • Caregiver: The commitment was to provide an update by nine ten, even if the arrangement was still unresolved.

04 • Dispatcher: You're right. I need to tell the client what is known rather than leave the missed callback unexplained.

05 • Caregiver: Please clarify whether the original arrival estimate has changed, and avoid presenting the backup request as confirmed.

06 • Dispatcher: The original estimate remains nine thirty to nine forty-five, but it is still subject to the current travel situation.

07 • Caregiver: I'll keep that distinction in my own message. Who will contact the client now and own the next update?

08 • Dispatcher: I will call now and agree the next contact time, following our approved process for this disruption.

09 • Caregiver: Thank you. I'll record the missed update and your follow-up without describing an unconfirmed cancellation.

10 • Dispatcher: That is accurate. Please let me know through the established route if another urgent concern emerges.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

A complaint after the visit

A client explains why a small delay felt more serious than the worker expected.

01 • Client: The delay itself was frustrating, but being told 'soon' three times made it harder to plan my morning.

02 • Caregiver: I understand. The vague updates left you without a useful time range or a clear person to contact.

03 • Client: Exactly. I didn't know whether to wait, call the office, or assume the visit had been cancelled.

04 • Caregiver: I'm sorry our communication created that uncertainty. I'll pass on the specific concern through the complaint process.

05 • Client: Please don't just say I was unhappy. Tell them what would have made the situation easier.

06 • Caregiver: You wanted an honest estimate, a named contact, and an update even if the arrangement was not yet confirmed.

07 • Client: Yes. I know delays happen, but I need to understand what is actually known at the time.

08 • Caregiver: I'll state that clearly. I can confirm that I will pass on the concern, but I can't promise the review outcome.

09 • Client: That's a fair distinction. How will I know the concern has reached the right person?

10 • Caregiver: We'll use the agency's acknowledgment process and confirm the route for any further follow-up with you.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Delays, complaints, and reliable follow-up

Your fictional 9:00 visit will be delayed. At 8:50, dispatch estimates arrival between 9:30 and 9:45 but cannot guarantee it. The client had planned breakfast support at 9:00. Dispatch is checking an approved backup arrangement and will update the client by 9:10.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are the client. You expected breakfast support at 9:00 and want a reliable next step, not another vague apology. You understand an estimate is not a guarantee.

Second round: At 9:10 no update has arrived. Use the approved follow-up route and describe the missed update accurately without assuming a backup was cancelled.

Scenario 2 | A worker cannot access the building

You arrive within the visit window, but the authorized entry method does not work and the client does not answer through the normal contact route. You do not know why. Your agency has a no-access procedure and a coordinator on duty.

Role A: Caregiver: report the access problem accurately and follow the approved procedure without inventing a reason or an unsafe entry method.

Role B: Coordinator: ask which approved contact steps were attempted and when; you will direct the next authorized action.

Possible opening: I'm at the scheduled address, but I cannot gain access through the approved entry method.

Success checks

- Give the time, location context, and actual access issue through the approved channel.
- Distinguish no response from a conclusion about the client's location or condition.
- Follow the agency's no-access and urgent-concern procedures without improvising entry.

Add a complication: A neighbor offers an unverified explanation and suggests an unofficial way inside. You must not treat either as authorization.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

BLS: Home Health and Personal Care Aides

<https://www.bls.gov/ooh/healthcare/home-health-aides-and-personal-care-aides.htm>

CDC: Plain Language Materials

<https://www.cdc.gov/health-literacy/php/develop-materials/plain-language.html>

BLS: Personal Care Aides Occupational Requirements

<https://www.bls.gov/ors/factsheet/pdf/personal-care-aides.pdf>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Home Care & Caregiving English

Professional audience: Home care aides, personal care assistants, and caregivers

Scope: Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

Dialogue 01: Sorting out a duplicated request

Original fictional setting: A caregiver and coordinator review a client's concern that laundry support has been forgotten.

Professional roles: Caregiver / Coordinator

Published dialogue (reference script, not my own performance):

Caregiver: Mrs. Vale says nobody handled her laundry yesterday. Today's assignment lists breakfast and light tidying, not laundry.

Coordinator: Yesterday's note says the washing was started, but it doesn't confirm whether the clothes were put away.

Caregiver: So we have a partial record, not evidence that the whole task was completed or missed.

Coordinator: That's right. Could you ask what she wants help with now without suggesting someone failed her?

Caregiver: She says there are clean clothes in the basket, and she would like them folded before visitors arrive.

Coordinator: That fits the light household support in today's plan, provided the other agreed needs can still be met.

Caregiver: We have forty minutes left, and breakfast is finished. I'll ask whether folding is her priority for that time.

Coordinator: Please also clarify that we are dealing with today's preference, not deciding what happened during yesterday's visit.

Caregiver: Understood. I'll describe the basket as I find it and avoid attributing the unfinished task to anyone.

Coordinator: If time runs short, tell her which agreed task remains and ask me about arranging follow-up support.

Caregiver: I'll confirm the priority with her now, then report the actual outcome before the visit closes.

Coordinator: Thank you. That gives us a clear account of the present need without turning an incomplete note into blame.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.
 2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
 3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
 4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
 5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.
- Use voice replies when available. If I prefer selections, offer four plausible spoken replies and explain my choice before continuing. Do not require written compositions.

END PROMPT

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

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Caregiver: I'll confirm the priority with her now, then report the actual outcome before the visit closes.

Coordinator: Thank you. That gives us a clear account of the present need without turning an incomplete note into blame.

END REFERENCE

TASK: Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus. Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.
2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.
3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.
4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.
5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT